

City General Hospital

Department of Neurosurgery & Neuro-Oncology

Patient Name:	Kavitha Reddy	MRN:	MGH-2024-09215
Age / Sex:	47 years / Female	Date:	05-Mar-2026
DOB:	22-Sep-1978	Physician:	Dr. Vikram Rao, MCh (Neurosurgery)

NEUROSURGICAL CONSULTATION REPORT

Chief Complaint: Progressive headaches, new-onset seizures, and right-sided weakness for 6 weeks.

History of Present Illness: Mrs. Kavitha Reddy is a 47-year-old female who presents with a 6-week history of severe, progressive headaches worse in the morning, two witnessed tonic-clonic seizures in the past 2 weeks, and progressive right-sided arm and leg weakness. She reports word-finding difficulty and personality changes noted by her family. No prior neurological history. No significant family history of brain tumors. Non-smoker, no alcohol consumption.

MRI BRAIN — RADIOLOGY REPORT

Modality: MRI Brain with and without contrast (3T scanner) — 05-Mar-2026

Findings: A large, irregular, heterogeneously enhancing mass lesion is identified in the LEFT TEMPORAL-PARIETAL region measuring approximately **4.8 cm × 4.2 cm × 3.9 cm**. The lesion demonstrates central areas of necrosis, surrounding vasogenic oedema extending into the adjacent white matter, and significant midline shift of **7 mm to the right**. Post-gadolinium images show ring-enhancement with an irregular inner border — highly characteristic of high-grade glioma. No evidence of leptomeningeal spread. Herniation not yet present. DWI shows restricted diffusion within the necrotic core.

Radiological Impression: Large ring-enhancing left temporal-parietal mass with mass effect, midline shift, and surrounding oedema. Features highly suggestive of Glioblastoma Multiforme (GBM), WHO Grade IV.

NEUROLOGICAL EXAMINATION

GCS: 14/15. Oriented to person and place. Right-sided hemiparesis (power 3/5 upper and lower limb). Expressive dysphasia noted. Fundoscopy: bilateral papilloedema consistent with raised intracranial pressure. Plantar reflex: extensor on right. Brisk deep tendon reflexes right side.

LABORATORY RESULTS

Test	Result	Normal Range	Unit	Flag
Haemoglobin	11.2	12.0 – 16.0	g/dL	LOW ↓
WBC Count	9800	4000 – 11000	cells/μL	Normal
Platelets	218	150 – 400	×10 ³ /μL	Normal
LDH	488	140 – 280	U/L	HIGH ↑
CRP	18.2	< 5.0	mg/L	HIGH ↑
Sodium	131	135 – 145	mEq/L	LOW ↓
Serum Creatinine	0.9	0.5 – 1.1	mg/dL	Normal
CEA (tumour marker)	3.2	< 5.0	ng/mL	Normal
AFP	1.8	< 10	ng/mL	Normal

CLINICAL NOTES & PLAN

MRI findings are highly consistent with Glioblastoma Multiforme (GBM), the most aggressive primary brain tumour (WHO Grade IV). The size (4.8 cm), ring-enhancement, necrotic core, and 7 mm midline shift indicate an advanced lesion. The hyponatraemia (131 mEq/L) is likely secondary to SIADH related to the intracranial mass. Elevated LDH indicates high tumour burden and poor prognosis. Immediate plan: Start IV Dexamethasone 8 mg BD for cerebral oedema. Levetiracetam 1000 mg BD for seizure prophylaxis. Refer to tumour board for surgical planning — maximal safe resection followed by concurrent Temozolomide chemotherapy and radiotherapy (Stupp protocol).

PRELIMINARY DIAGNOSIS

Glioblastoma Multiforme (GBM) — Left Temporal-Parietal Region, WHO Grade IV, 4.8 cm with 7 mm midline shift.